

PACKAGE INSERT

FASTUM GEL

1. COMPOSITION

100 g of Gel Contain:

Active ingredient: ketoprofen 2.50 g

Excipients: carbomer , ethanol, neroli fragrance, lavandin fragrance, triethanolamine, purified water.

2. PHARMACOLOGICAL PROPERTIES

Ketoprofen, in a suitable vehicle, reaches by transcutaneous route the inflammation site, allowing local treatment of pain of joints, tendons, ligaments and muscles.

Following oral administration of a single dose within 2 hours maximum blood concentrations are reached.

Plasma half-life of ketoprofen varies from 1 hour to 3 hours: binding to plasma proteins is 60-90%. Elimination occurs mainly by urinary route, in glucuronoconjugated form: approximately 90% of the administered dose is excreted within 24 hours.

Conversely the absorption by cutaneous route is very low. In fact the application of 50-150 mg of ketoprofen by percutaneous route determines plasma levels of the active ingredient equal to 0.08-0.15 µg/ml about 5-8 hours after the application.

3. INDICATIONS

Local treatment of painful disorders of the osteo-articular and muscular system of rheumatic or traumatic origin: contusions, distortions, muscle strains, stiff neck, lumbago.

4. SIDE EFFECTS

There have been reports of localised skin reactions which might subsequently spread beyond the area of application. Cases of more severe reactions such as bullous or phlyctenular eczema which may spread or become generalized have occurred rarely.

Other systemic effects of NSAIDs: these depend on the transdermal diffusion of the active ingredient, and thus on the amount of gel applied, the area involved, the degree of the skin integrity, the duration of the treatment and the use of occlusive dressings (digestive and kidney effects).

The following CIOMS frequencies have been used: very common ($\geq 1/10$); common

($\geq 1/100$, $< 1/10$); uncommon ($\geq 1/1000$, $< 1/100$); rare ($\geq 1/10,000$, $< 1/1000$); very rare ($< 1/10,000$); not known (the frequency cannot be established based on the available data).

Immune system disorders

Not known: anaphylactic reactions, including anaphylactic shock, angioedema, hypersensitivity reactions.

Skin and subcutaneous tissue disorders

Uncommon: localised skin reactions such as erythema, eczema, pruritus and burning sensation.

Rare: dermatological reactions (photosensitisation, and urticaria).

Cases of more severe adverse reactions, such as bullous or phlyctenular eczema which may spread beyond the area of application or become generalised, have occurred rarely.

Not known: bullous dermatitis.

Renal and urinary disorders

Very rare: new cases or worsening of existing cases of renal insufficiency.

Elderly patients are particularly susceptible to the adverse effects of non-steroidal anti-inflammatory drugs.

5. PRECAUTIONS AND WARNINGS

- The gel should be used in caution in patients with reduced heart, liver or kidney function:

Isolated cases of systemic adverse reactions affecting the kidneys have been reported.

- The gel should not be used with occlusive dressings.
- The gel must not come into contact with the mucous membranes and with the eyes.
- The treatment should be interrupted if rash appears.
- Treatment should be discontinued immediately upon development of any skin reactions, including cutaneous reactions after co-application of octocrylene containing products.
- It is recommended to protect treated areas by wearing clothing during all the application of the product and two weeks following its discontinuation to avoid the risk of photosensitisation. Hands should be washed thoroughly after each application of the product.
- The recommended treatment length should not be exceeded due to the risk of developing contact dermatitis and of an increase in photosensitivity reactions over time.

- Patients with asthma combined with chronic rhinitis, chronic sinusitis, and/or nasal polyposis have a higher risk of allergies to aspirin and/or NSAIDs than the rest of the population.

6. CONTRAINDICATIONS

- History of hypersensitivity to one of the excipients
- History of any photosensitivity reaction.
- Known hypersensitivity reactions, such as asthma symptoms, allergic rhinitis to ketoprofen, fenofibrate, tiaprofenic acid, acetylsalicylic acid or other NSAIDs. History of skin allergy to ketoprofen, tiaprofenic acid, fenofibrate, UV blockers or perfumes
- Sun exposure, even in case of hazy sun, including UV light from solarium, during the treatment and 2 weeks after its discontinuation (see section “Special warning and precautions for use”).
- Application on pathological skin changes such as eczema or acne; in infectious skin or open wounds.
- Third trimester of pregnancy (see section “Pregnancy”)

7. DRUG AND SUBSTANCE INTERACTIONS

Interactions are unlikely as serum concentrations after topical administration are low. .

8. PREGNANCY AND LACTATION

As there has been no such experience with the topical formulation, the following is stated according to the systemic formulation:

8.1 Pregnancy

In mice and rats, there is no evidence of teratogenic or embryotoxicity. In the rabbit, slight embryotoxicity likely related to maternal toxicity has been reported. There are no clinical data from the use of topical forms of ketoprofen during pregnancy. Even if systemic exposure is lower compared with oral administration, it is not known if the systemic ketoprofen exposure reached after topical administration can be harmful to an embryo/fetus. During the first and second trimester of pregnancy, Fastum should not be used unless clearly necessary. If used, the dose should be kept as low and duration of treatment as short as possible.

During the third trimester of pregnancy, systemic use of prostaglandin synthetase inhibitors including ketoprofen may induce cardiopulmonary and renal toxicity in the fetus. At the end of the pregnancy prolonged bleeding time in both mother and child may occur, and labour can be delayed. Therefore, Fastum is contraindicated during the last trimester of pregnancy (see Section “Contraindications”)

8.2 Lactation

No data are available on the excretion of ketoprofen in human milk. Ketoprofen is not recommended in nursing women.

9. DRIVING AND USING MACHINES

None known.

10. DOSAGE AND ADMINISTRATION

Apply a thin layer of gel 1 to 3 times daily onto the area of affected skin, gently massaging to help absorption.

Special population

Pediatric population

The safety and efficacy of ketoprofen gel in children have not been established.

11. INSTRUCTION FOR USE

Opening of the soft tube: unscrew the cap and perforate the diaphragm of aluminium with the point of the inverted cap.

Pre-filling of the dispensing tube: push the dispenser cap several times or push the base of the tube until the gel appears; it is advisable to use it in a horizontal position.

12. OVERDOSAGE

Overdose is unlikely to be caused with topical administration. If accidentally ingested, the gel may cause systemic undesirable effects depending on the amount ingested. However, if this occurs, treatment will be symptomatic and supportive as in cases of overdose of oral-inflammatories.

13. STORAGE CONDITIONS

Store in a cool and dry place, below 30°C

14. SHELF-LIFE

Tube: 48 months

Dispenser: 36 months

15. DESCRIPTION OF PRODUCT

Soft aluminium tube, treated inside with non-toxic epoxyresin, containing 10 g, 30 g, 50 g.
Or

Pump dispenser: rigid polypropylene dispenser containing 50 g

Not all pack sizes may be marketed.

16. DOSAGE FORMS AVAILABLE

2.5% gel for external use. Tube of 10 g, 30 g and 50 g. Tube with dispenser of 50 g
Not all pack sizes may be marketed.

17. MANUFACTURER

A. Menarini Manufacturing Logistics and Services S.r.l.
Via Sette Santi, 3 - Florence, Italy

18. Date of last revision

January 2023